



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : حنان محمد السيد هلال
المحافظة : الجيزة
الحى / المدينة : الاختصاص
رقم البطاقة : 27004171500882 رقم التليفون : 01094817354
العنوان : سى
العمر (تاريخ الميلاد) : 17-04-1970 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 07-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: حنان محمد السيد هلال National ID: 27004171500882	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

حنان
محمد السيد هلال

٩ حي الشباب - المجاورة ١
٦ أكتوبر - الجيزة

٢٧٠٠٤١٧١٥٠٠٨٨٢

JE0750672

٢٠٢١/١٢ ٢٧٠٠٤١٧١٥٠٠٨٨٢

حاصلة على ليسانس حقوق

أنثى مسلمة متزوجة

صلاح الدين ابراهيم محمد عطا الله

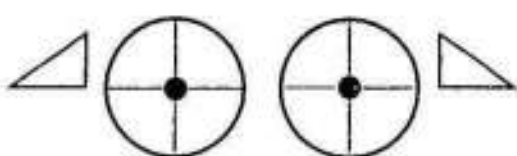
٢٠٢٨/١٢/٠٨ البطاقة سارية حتى

١

MDT FORM 24 / 6 / 2025 G

Name	حسان محمد السيد هادي	MRN	٣١٨٢ / ٢٤
Age	55yr	ID	
Menopausal status		Phone	١١٩٤٨١٧٣٥٤
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	part of
CV - - ٤١٧١٥ - - ٨٨٢			

Clinical Data

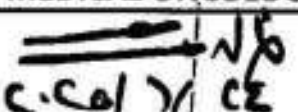
BX: IDC	R			L		
ER: 0/8	PR: 3/8	HER 2: Amplified	KI67: 60%			
Sonomammography: by SISA						
Mets Workup:				TNM		

Recommendations: post MRM Post Chemo
No Residual
Five LNs (0/14)
PCR
For possibility of Adj HT

MDT OUTCOME & PLAN

① PORT
② Capecitabine up to 1yr. 18/6/2025

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Name: حنان محمد السيد هلال			
MR. : 5677			
Detailed Medical Report			
			السيد
تجربة طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient was initiated on neoadjuvant chemotherapy with taxol, receiving 140 mg in week 1 and continuing in weeks 2 and 3 as documented. Additional pharmacy orders included wt 84, ht 150, and sa 1.8; laboratory studies were pending, and a non-contrast chest CT was scheduled for 3 February 2026. The patient was also scheduled for a free breast and axillary work-up on the same date. The patient has completed taxol weeks 1 through 3. The patient received an adjustment to the PHESGO regimen, noted as C16, with APUS and BSM administered in April 2026; the treatment was provided free of charge. The patient has completed 16 cycles of PHESGO. The patient was noted to be free for a brain MRI on 7 April 2026 while awaiting approval for continuation of PHESGO therapy. No additional treatment cycles were specified in this entry. The patient's pathology was reported as left breast cancer, clinical stage T2 N3, with IDC, ER 0/8, PR 3/8, HER2 2+, and a recurrence score of 68 %. Neoadjuvant chemotherapy was recommended on 31 May 2025. Imaging included left MRM and CT, with findings of cryptic YPN 0/14. The patient received PHESGO in cycle 17 and a brain MRI was performed free of charge in April 2026. The patient has completed 17 cycles of PHESGO.			
تحريرا في ,			
		التاريخ :	19-09-2026
		الوقت :	
		هنا اسامه عبد الرحيم عبد البر	الطبيب المعالج :
يعتمد ,			

Patient Name : الأستاذة / حنان محمد السيد هلال

Patient No. : 270041715C0882

Path No. : SR-1401/24

Sex : Female

Age : 54 Y

Referring Doc :

Request No. : 13002664

Request Date : 23/09/2024

Verification Date : 01/10/2024

Financial Category : نفقة التولية - اورام دار السلام

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Slide revision.

Gross Examination:

- One paraffin block labeled by (11079/24) and one slide were received.

Microscopic Examination:

* Histological type: Invasive duct carcinoma.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).

* DCIS: Negative.

* Angioinvasion: Negative.

* Perineural invasion: Negative.

* Microcalcification: Negative.

Diagnosis:

Left breast cancer, Slide revision:

- Invasive duct carcinoma, Grade II.

- For further IHC (ER, PR, Her2, Ki67)

Signature

Dr. Walaa Ghonaim.

Dr. Toka Medhat.

Verified by :

DR.Nermeen Mohammed

Printing Date : 22/10/2024 10:06:50

Page 1 of 1

Printing By : marwa mohamed youss

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / حنان محمد السيد هلال	Request No.	: 13002664
Patient No.	: 27004171500882	Request Date	: 23/09/2024
Path No.	: SR-1401/24	Verification Date	: 13/10/2024
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer.

Nature of Specimen: Slide revision.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):

ER: No nuclear reaction in tumor cells (score 0/8).

PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).

Her2: Moderate complete membrane reaction in >10% of tumor cells (score +2) for SISH.

Ki67: 60% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Slide revision.

Invasive duct carcinoma Gradell.

PT:

ER: Negative (score 0/8).

PR: Positive (score 3/8).

Her2: Equivocal (score +2) for SISH.

Ki67: Positive in 60% of tumor cells.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :

DR.Menna Nabil

Printing Date : 22/10/2024 10:06:57

Page 1 of 2

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CAP

Patient Name الأستاذ / حنان محمد السيد هلال
Patient No. 27004171500882
Path No. SR-1401/24
Sex Female
Age 54 Y
Referring Doc.

Request No. 13002664
Request Date 23/09/2024
Verification Date 13/10/2024
Financial Category أورام دار السلام - نفقة الدولة
Department Out Patient

Registered by marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ghoneim.

Dr. Sagda Elkady.

Verified by :
DR. Menna Nabil

Printing Date : 22/10/2024 10:06:57

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Printed By : marwa mohamed yousse

Patient Name : الأستاذة / حنان محمد السيد هلال

Patient No. : 27004171500882

Path No. : 1971/25

Sex : Female

Age : 55 Y

Referring Doc :

Request No. : 13009217

Request Date : 31/05/2025

Verification Date : 18/06/2025

Financial Category : لورام دار السلام - نفقة الدولة

Department : Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy.

Gross Examination:

- * Procedure: Left modified radical mastectomy specimen measured 23x18x6 cm, with covering skin measured 18x15 cm.
- * Tumor extent: A greyish white tumor bed site.
- * Size: 8x7x3 cm.
- * N & A: Looked unremarkable grossly.
- * Distance from margins: 3 cm away from the least deep margin.
- * Axillary lymph nodes: Axillary fat measured 9x7 cm, entangling 14 lymph nodes, up to 2x1.5x0.5 cm.

* 13 Blocks:

1:8- T.B.S., 9- N&A., 10- Deep margin., 11:13- LNs

- Gross was done by: Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed tumor.
- Cellularity (0%).
- * Angioinvasion: Not detected.
- * Ductal carcinoma in situ: Not detected.
- * Nipple & areola: Free.
- * Deep Margin: Free.
- * Axillary lymph nodes: 14 lymph nodes free of viable metastatic deposits, LNs (0/14).
- * Histologic response to therapy: Pathological complete response to therapy.

Verified by :

Dr. Duaa Mohamed

Patient Name : الأستاذة / حنان محمد السيد هلال
Patient No. : 27004171500882
Path No. : 1971/25
Sex : Female
Age : 55 Y
Referring Doc :

Request No. : 13009217
Request Date : 31/05/2025
Verification Date : 18/03/2025
Financial Category : أورام - فئة التمالة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Diagnosis:

Left breast cancer (Post-NACT), Left modified radical mastectomy:

- No residual viable remnants of the previously diagnosed tumor.
- No DCIS.
- Deep Margin: Free.
- Nipple & areola: Free.
- Axillary lymph nodes: Free 14 lymph nodes, LNs (0/14).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature

Dr. Walaa Ibrahim Ghor m.
Dr. Heba Hassan Adly

Verified by :

Dr. Duaa Mohamed

Printing Date : 18/06/2025 10:46:26

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Women's Health Outreach Program

Name: Hanan Mohammed Abdel Hamid Ibrahim

Date: 9/10/2024

Contrast Enhanced Digital Mammography

Clinical summary: 56years old female with left pathologically proved ILC II

Technique

MLO and CC views (low energy & recombined post contrast)

Targeted High resolution ultrasound

Detailed Findings:

Density: ACR B

Background/Uptake: minimal

Right breast

- Low energy (non contrast images) show : no masses, asymmetry, distortion or suspicious calcifications.
- Scattered amorphous /punctate calcification without suspicious distribution
- Intramammry lymph node is noted
- Recombined (post contrast) images show: no pathological uptake.
- Ultrasound demonstrated no solid or cystic masses.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Low energy (non contrast images) show : global asymmetrical densities mainly centrally with overlying pleomorphic calcifications with segmental and regional distribution.
- Recombined (post contrast) images show: diffuse scattered confluent moderate heterogeneous enhancing masses(ranging 5mm to 22mm) seen extend from nipple to the posterior third of the breast.
- The overall coverage area 14x12.6x8.7cm , this correspond to the calcification distribution
- Ultrasound : Diffuse pathological process involve whole the breast in form of altered echogenicity , dilated ducts and calcifications.
- Pathologically enlarged level I axillary lymphnodes, all are discrete , more than 3, the largest shows diffuse cortical thickness up to 7mm with indenting hilum and overall measurement 9x8mm .



Patient Name: Hanan Mohamed Al Sayed Helal	Date : 18/09/2024
Exam : C.T CHEST	Age : 54 Year(s)
Medical ID: 3182/2024	Pat. ID: 79184

Multislice non contrast CT scan of the chest, abdomen and pelvis revealed:

I-CHEST:

- Left breast ill-defined small nodular parenchymal soft tissue lesions. Enlarged ipsilateral axillary (level I) lymph nodes, the largest measures about 2 x 2.6 cm.
- Right upper lung lobe apical small nodule measures about 4 mm.
- No pulmonary consolidation, cavitation or bronchiectatic changes.
- No CT evidence of pathological hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- Mildly increased cardiac size.
- Enlarged right thyroid lobe showing hypodense nodule. For US correlation.

II-ABDOMEN & PELVIS:

- Enlarged liver showing homogenous parenchymal pattern. No hepatic lesions on non-contrast basis or biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, suprarenal glands, kidneys.
- Average filling of the urinary bladder showing no sizable diverticular outpouchings.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- D12 vertebral body right aspect small lytic lesion with right iliac wing sclerotic lesion measuring 7 mm
- Right femoral head sclerotic focus mostly bone island.
- No sizable destructive osseous lesions.

OPINION:

- Right upper lung lobe apical small nodule, for follow-up.
- D12 vertebral body small lytic lesion and right iliac wing sclerotic focus. For bone scan correlation.
- No sizable hepatic lesions on non-contrast basis.

Dr. Ali Elsisy, MSc.

Dr. Mohamed Mohsen, M.D.

Patient Name: Hanan Mohamed Al Saeid

Date: 15/10/2024

Referring physician: Prof.

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- No definite active hot spot that could account for osseous metastatic lesions.
- Spondylodegenerative changes are seen at dorso / lumbar vertebrae.
- Osteoarthritic changes are seen at both shoulders,
- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is seen free of definite osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR. Sherin Mahdy, MD

DR Mohamed Emam

- Two enlarged left supraclavicular lymphnodes, the largest show non uniform cortical thickness up to 6mm with overall measurement 9x8mm

Impression:

Right breast (BIRADS 2): benign .

Left breast (BIRADS 6): multicentric pathological enhancing process with finding as described.

DR, Marwa Ramzy, FRCR



MS
رئيس